

Neuroablative Techniques for Chronic Pain



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Medicare Advantage Medical Coverage Policy

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Related Medicare Advantage Medical/Pharmacy Coverage Policies

[Headache and Occipital Neuralgia Treatments](#)
[Injections for Chronic Pain Conditions](#)

Related CMS Documents

Please refer to [CMS Medicare Coverage Database](#) for the most current applicable CMS National Coverage Determination (NCD)/Local Coverage Determination (LCD)/Local Coverage Article (LCA). Refer to CMS website for the most current applicable [CMS Online Manual System \(IOMs\)](#) and [Transmittals](#).

Humana and subsidiaries apply a Medicare Administrative Contractor's (MAC's) LCD to all requests (both Part A and Part B services) within the MAC's applicable geographic jurisdiction.

Type	Title	Document ID Number	Jurisdiction Medicare Administrative Contractors (MACs)	Applicable States/Territories
NCD	Induced Lesions of Nerve Tracts	160.1		
LCD LCA	Facet Joint Interventions for Pain Management	L38841 A58477	J5 - Wisconsin Physicians Service Insurance Corporation (Part A/B MAC)	IA, KS, MO, NE
	Sacroiliac Joint Injections and Procedures	L39475 A59257	J8 - Wisconsin Physicians Service Insurance Corporation (Part A/B MAC)	IN, MI
LCD LCA	Billing and Coding: Cryoneurolysis Instructions	A60507	J6 – Wellpoint Federal (Part A/B MAC)	IL, MN, WI
	Facet Joint Interventions for Pain Management	L35936 A57826	JK – Wellpoint Federal (Part A/B MAC)	CT, MA, ME, NH, NY, RI, VT
	Peripheral Nerve Blocks	L36850 A57452		
	Sacroiliac Joint Injections and Procedures	L39455 A59233		
	Thermal Destruction of the Intraosseous Basivertebral Nerve (BVN) for Vertebrogenic Lower Back Pain	L40302 A60324		
LCD LCA	Facet Joint Interventions for Pain Management	L38773 A58364	J15 - CGS Administrators, LLC (Part A/B MAC)	KY, OH
	Sacroiliac Joint Injections and Procedures	L39383 A59154		
LCD LCA	Billing and Coding: Cryoneurolysis Instructions	A59752	JE - Noridian Healthcare Solutions, LLC (Part A/B MAC)	CA, HI, NV, American Samoa, Guam, Northern Mariana Islands
	Facet Joint Interventions for Pain Management	L38801 A58403	JF - Noridian Healthcare	AK, AZ, ID, MT, ND, OR, SD, UT, WA, WY

	Injections – Tendon, Ligament, Ganglion Cyst, Tunnel Syndromes and Morton’s Neuroma Intraosseous Basivertebral Nerve Ablation Nerve Blockade for Treatment of Chronic Pain and Neuropathy Sacroiliac Joint Injections and Procedures	L34218 A57079 L39642 A59466 L35456 A56034 L39462 A59244	Solutions, LLC (Part A/B MAC)	
LCD LCA	Facet Joint Interventions for Pain Management	L34892 A56670	JH - Novitas Solutions, Inc. (Part A/B MAC) JL - Novitas Solutions, Inc. (Part A/B MAC)	AR, CO, LA, MS, NM, OK, TX DE, DC, MD, NJ, PA
LCD LCA	Facet Joint Interventions for Pain Management Sacroiliac Joint Injections and Procedures Thermal Destruction of the Intraosseous Basivertebral Nerve (BVN) for Vertebroгенic Lower Back Pain	L38765 A58350 L39402 A59192 L39420 A59205	JJ - Palmetto GBA (Part A/B MAC) JM - Palmetto GBA (Part A/B MAC)	AL, GA, TN NC, SC, VA, WV
LCD LCA	Facet Joint Interventions for Pain Management Peripheral Nerve Blocks	L33930 A57787 L33933 A57788	JN - First Coast Service Options, Inc. (Part A/B MAC)	FL, PR, US VI

Description

Neuroablative techniques in pain management consist of several surgical and non-surgical methods to denervate a nerve. The goal of denervation is to interrupt the pain signals that are sent to the brain from the joints and nerves. An additional objective is to reduce the likelihood of, or to delay, any recurrence by selectively destroying pain fibers without causing excessive sensory loss, motor dysfunction or other complications.

Many techniques accomplish denervation including, but may not be limited to:

- **Chemical neurolysis**, also known as chemical ablation, chemical denervation or chemodenervation, involves the injection of neurolytic agents (eg, alcohol, hypertonic saline, phenol). This proposed treatment option for chronic pain generally results in a permanent ablation of the nerve.
- **Cooled radiofrequency denervation** is a modification of conventional radiofrequency ablation (RFA), in that it maintains the tissue temperature immediately adjacent to the electrode at 60°C while the target tissue (nerve) is heated to 75°C or higher. This purportedly allows for a larger volume of treated tissue without the risk of damage to the adjacent tissue. Examples of devices used for this procedure include, but may not be limited to, the **Accurian RF Platform** (when used in the cooled RF mode), **COOLIEF Cooled RF Probe** and **Coolief Sinergy** (Coolief Sinergy is specifically for the sacroiliac joint).
- **Cryosurgery** may also be referred to as cryoablation or cryodenervation and is a technique of using extreme cold to destroy tissue, which is cooled to below -20°C by a probe circulating liquid nitrogen.
- **Cryotherapy**, similar to cryosurgery, uses extreme cold to destroy tissue, but is generally used to specifically target peripheral nerves. An example of cryotherapy is **CryoNB (cryo nerve block) therapy**, which is performed with the **CryoICE cryoSPHERE cryoablation probe** and is proposed as a method for blocking postoperative pain by temporarily ablating peripheral nerves.
- **Facet denervation**, one of the most commonly performed neuroablative procedures, involves destruction or interruption of a facet joint nerve (medial branch nerve) to relieve chronic pain in the cervical, thoracic or lumbar spine regions.
- **Intrasept Intraosseous Nerve Ablation System** is a specialized radiofrequency ablation device, which has been granted US Food & Drug Administration (FDA) approval strictly for destruction of the basivertebral nerve of the L3-S1 vertebrae. It is proposed as a treatment option for chronic low back pain.
- **iovera® System** (also referred to as percutaneous cryoablation) is similar to cryosurgery, in that it uses cold to ablate a peripheral sensory nerve, but it is done in such a manner that it only produces a temporary denervation and subsequent interruption of the pain signals. It is purported that the nerves will slowly regenerate and normal nerve conduction resumes once the nerve function is restored. This treatment may also be described as a cold injection or an iovera® injection. It has been suggested as a possible treatment for knee pain associated with osteoarthritis or for managing postoperative pain following total knee arthroplasty (replacement).
- **Laser ablation** is proposed as a noninvasive treatment which uses laser energy to ablate a peripheral nerve.
- **Pulsed radiofrequency denervation** is another proposed alternative to traditional radiofrequency neurotomy. It delivers short bursts of radiofrequency current instead of a continuous flow, which allows the needle to remain relatively cool so that the tissue temperature decreases slightly between each burst, reducing the risk of destroying nearby tissue. Examples of devices used for this procedure include,

but may not be limited to, the **Accurian RF Platform, IonicRF Generator, MultiGen 2 RF Generator System** or **NeuroTherm NT2000IX** (when any of these devices are used in pulsed mode).

- **Radiofrequency ablation (RFA)** also referred to as be referred to as nonpulsed radiofrequency ablation, percutaneous radiofrequency neuroablation, radiofrequency coagulation, radiofrequency denervation, radiofrequency lesioning, radiofrequency neuroablation, radiofrequency neurotomy or rhizotomy (articular rhizolysis). This percutaneous procedure utilizes radiofrequency current/energy to heat and ablate/denervate the target nerve. This technique involves the constant application of energy, usually at 80-85°C via an image-guided needle electrode inserted through the skin to the affected nerve. Examples of devices used for RFA include, but may not be limited to, the **Baylis Pain Management Radiofrequency Generator** and **G4 RF Generator**.

This policy addresses neuroablative/denervation (rhizotomy) procedures only, and should be distinguished from *intradiscal* electrothermal procedures, which is a treatment for back pain that applies heat to the *disc* or *disc wall*.

Coverage Determination

Humana and subsidiaries follow the Medicare requirements that only allow coverage and payment for services that are reasonable and necessary for the diagnosis and treatment of illness or injury or to improve the functioning of a malformed body member except as specifically allowed by Medicare.

Humana and subsidiaries apply any applicable National Coverage Determination (NCD) and any applicable Local Coverage Determinations (LCDs) to the services and jurisdiction at issue. See the Related CMS Documents Section above for any such NCDs or LCDs.

Facet Joint Denervation (64633, 64634, 64635, 64636)

Please refer to the above CMS guidance for **facet joint denervation** (facet joint interventions).

Basivertebral Nerve Denervation (64628, 64629)

The following criteria apply to jurisdictions without an LCD (J5/J8, J15, JH/JL, JN).

Humana and subsidiaries determine medical necessity for **denervation of the intraosseous basivertebral nerve via radiofrequency ablation** for the treatment of chronic low back pain based on the criteria contained in LCD – Thermal Destruction of the Intraosseous Basivertebral Nerve (BVN) for Vertebrogenic Lower Back Pain (L39420).

In order to determine medical necessity consistently, Humana and subsidiaries may consider the criteria contained in the following:

The following criteria apply to all jurisdictions.

Morton's Neuroma Denervation (64632)

Denervation of a Morton's neuroma via a neurolytic agent will be considered medically reasonable and necessary when ALL of the following requirements are met:

- Diagnosis of a swollen, inflamed nerve in the ball of the foot (eg, Morton's neuroma, Heuter's neuroma, Hauser's neuroma or Iselin's neuroma)²⁶; **AND**
- The diagnosis has been confirmed with an injection of local anesthetics and/or steroids²⁶

Trigeminal Nerve Denervation (64600, 64605, 64610)

Denervation of the trigeminal nerve via nonpulsed radiofrequency ablation will be considered medically reasonable and necessary when ALL of the following requirements are met:

- Diagnosis of trigeminal neuralgia¹¹²; **AND**
- Failure of conservative treatment (pharmacological) under the direction of a healthcare professional, or inability to tolerate side effects of the medication(s)¹¹²

The use of the criteria above provides clinical benefits highly likely to outweigh any clinical harms (eg, adverse effects including, but not limited to, a mild burning sensation in the treatment dermatome, which may subside after 1-3 weeks; hypoesthesia may occur but generally resolves within 3-4 months; risk of bleeding, infection, dural puncture or spinal cord trauma⁴²). Services that do not meet the criteria above are not medically reasonable and necessary and may result in unnecessary exposure to potential complications. Medically unnecessary services carry risks of adverse outcomes and may interfere with the pursuit of other treatments which have demonstrated efficacy.

Coverage Limitations

[US Government Publishing Office. Electronic code of federal regulations: part 411 – 42 CFR § 411.15 - Particular services excluded from coverage](#)

The following criteria apply to all jurisdictions.

The following **ablative techniques** will **not** be considered medically reasonable and necessary:

- Cooled radiofrequency denervation^{45,46,72,73}; **OR**
- Endoscopic radiofrequency ablation/rhizotomy⁶⁰ for any indication including, but not limited to, the facet joint/nerve; **OR**
- Laser ablation¹⁸⁻²⁵; **OR**
- Percutaneous cryoneurolysis (eg, the iovera[®] System) (**64640, 0440T, 0041T, 0442T, C9809**) for any indication including, but not limited to, knee osteoarthritis or before/during/after total knee replacement surgery^{49,69,70,102}; **OR**

- Pulsed radiofrequency denervation, for any indication^{72,76,77,78}; **OR**
- Radiofrequency ablation/neuroablative denervation for the treatment of **chronic pain** including, but may not be limited to, the following conditions/areas of the body/nerves **regardless of the type of neuroablative technique used**:
 - Coccygodynia (coccydynia)⁷⁴; **OR**
 - Dorsal root ganglia¹¹⁰; **OR**
 - Genicular nerve **(64624)**^{3,79,96,106}; **OR**
 - Hip, knee or pelvic/pelvis osteoarthritis/pain^{40,45,46,73,79}; **OR**
 - Nerves innervating the sacroiliac joint (SIJ) (the dorsal sacral rami lateral branch) **(64625)**³⁰⁻³⁵; **OR**
 - Post herniorrhaphy groin pain¹⁰⁹; **OR**
 - Pudendal nerve **(64630)**^{56,68}; **OR**
 - Sacroiliac joint (SIJ) **(64625)**³⁰⁻³⁵; **OR**
 - Shoulder pain (suprascapular nerve) **(64640)**^{40,78}; **OR**
 - Sural nerve for ankle pain **(64640)**¹; **OR**
 - Terminal (peripheral) nerve ending **(64640)**⁹¹

A review of the current medical literature shows that the **evidence is insufficient** to determine that these services are standard medical treatments. There is an absence of current, widely-used treatment guidelines or acceptable clinical literature (as defined by CMS) examining benefit and long-term clinical outcomes establishing the value of these services in clinical management.

Coding Information

Any codes listed on this policy are for informational purposes only. Do not rely on the accuracy and inclusion of specific codes. Inclusion of a code does not guarantee coverage and/or reimbursement for a service or procedure.

CPT® Code(s)	Description	Comments
64600	Destruction by neurolytic agent, trigeminal nerve; supraorbital, infraorbital, mental, or inferior alveolar branch	
64605	Destruction by neurolytic agent, trigeminal nerve; second and third division branches at foramen ovale	
64610	Destruction by neurolytic agent, trigeminal nerve; second and third division branches at foramen ovale under radiologic monitoring	
64624	Destruction by neurolytic agent, genicular nerve branches including imaging guidance, when performed	

64625	Radiofrequency ablation, nerves innervating the sacroiliac joint, with image guidance (ie, fluoroscopy or computed tomography)	
64628	Thermal destruction of intraosseous basivertebral nerve, including all imaging guidance; first 2 vertebral bodies, lumbar or sacral	
64629	Thermal destruction of intraosseous basivertebral nerve, including all imaging guidance; each additional vertebral body, lumbar or sacral (List separately in addition to code for primary procedure)	
64630	Destruction by neurolytic agent; pudendal nerve	
64632	Destruction by neurolytic agent; plantar common digital nerve	
64633	Destruction by neurolytic agent, paravertebral facet joint nerve(s), with imaging guidance (fluoroscopy or CT); cervical or thoracic, single facet joint	
64634	Destruction by neurolytic agent, paravertebral facet joint nerve(s), with imaging guidance (fluoroscopy or CT); cervical or thoracic, each additional facet joint (List separately in addition to code for primary procedure)	
64635	Destruction by neurolytic agent, paravertebral facet joint nerve(s), with imaging guidance (fluoroscopy or CT); lumbar or sacral, single facet joint	
64636	Destruction by neurolytic agent, paravertebral facet joint nerve(s), with imaging guidance (fluoroscopy or CT); lumbar or sacral, each additional facet joint (List separately in addition to code for primary procedure)	
64640	Destruction by neurolytic agent; other peripheral nerve or branch	
64999	Unlisted procedure, nervous system	
CPT® Category III Code(s)	Description	Comments
0440T	Ablation, percutaneous, cryoablation, includes imaging guidance; upper extremity distal/peripheral nerve	
0441T	Ablation, percutaneous, cryoablation, includes imaging guidance; lower extremity distal/peripheral nerve	
0442T	Ablation, percutaneous, cryoablation, includes imaging guidance; nerve plexus or other truncal nerve (eg, brachial plexus, pudendal nerve)	

HCPCS Code(s)	Description	Comments
C2618	Probe/needle, cryoablation	
C9809	Cryoablation needle (eg, iovera system), including needle/tip and all disposable system components, nonopioid medical device (must be a qualifying Medicare nonopioid medical device for postsurgical pain relief in accordance with Section 4135 of the CAA, 2023)	

Summary of Evidence

Cooled Radiofrequency Denervation

COOLIEF Cooled RF Probe and Coolief Sinergy

A report from an independent technology assessment organization⁷³ evaluating the Coolief system for osteoarthritis of the knee (KOA) found that the overall quality of the body of evidence for pain associated with KOA that is refractory to conservative measures was rated as very low. While studies generally demonstrate a reduction in pain from baseline up to 6 months, the clinical significance of this reduction is not consistently demonstrated. In addition, the lack of comparison with other minimally invasive techniques limits conclusions that can be drawn. A lack of long-term follow-up further inhibits conclusions that can be drawn regarding the durability of the effect of cooled RFA (CRFA). The authors further conclude that the overall quality of the body of evidence for the application of CRFA with the Coolief system prior to TKA was very low due to the inclusion of 1 eligible study. An evidence base of one moderate-sized study provides an insufficient quantity of evidence to inform evidence-based conclusions.

A different independent technology assessment organization⁴⁶ reports that the evidence is inconclusive due to very-low-quality comparative data when reviewing the Coolief system for KOA, noting study limitations include lack of blinding as well as short follow-up, and lack of studies comparing CRFA with standard RFA.

A report from an independent technology assessment organization⁷² also looked at cooled RF as a treatment option for chronic low back pain arising from the SI joint, noting an overall low-quality body of evidence evaluating CRFA that suggests it may be safe and potentially effective. The author also notes, though, that it is uncertain whether it is effective in the longer term and what effect it may have on quality of life.

An independent technology assessment organization⁴⁵ determined that the evidence for the Coolief system for treating hip pain is inconclusive due to too few data on the outcomes of interest. The organization found a small case series that suggests it is safe and reduces pain in chronic hip joint pain, but notes that the evidence is too limited in quantity and quality to permit conclusions. It was noted that this puts the study at high risk of bias due to small sample size, retrospective design, single-center focus and lack of control/comparison groups.

Endoscopic Radiofrequency Ablation/Rhizotomy

An independent technology assessment organization⁶⁰ provides a review of the literature for endoscopic rhizotomy, finding only one randomized controlled trial (RCT) evaluating the procedure compared to percutaneous RFA, as well as three studies comparing it with RF, cooled RFA or conservative treatment. All were of relatively small studies, the largest being 72 participants; all were also of somewhat short-term follow-up, with the longest being 2 years. Additional limitations of lack of randomization in three of the studies were noted to have the potential for bias.

iovera® System

An independent technology assessment organization⁷⁰ notes the use of the iovera system for pain associated with total knee arthroplasty (TKA) has a minimal level of support in the review of full-text clinical studies, as well as systematic reviews. Three clinical studies of poor or very-poor quality were identified, none of which reports clear benefits in pain, function or quality of life across all follow-up visits, although they do note lower opioid consumption in at least some of their analysis.

The same independent technology assessment organization⁶⁹ also reviewed the use of this device for treatment of knee osteoarthritis and, as with use for TKA, found minimal level of support in both the clinical studies and systematic reviews. Here it was noted that while there was statistically significant reduction in pain and improvement in function over sham treatment at 90-day follow-up, there was no benefit at 120 and 180 days, suggesting a potential placebo effect. An additional limitation was identified, noting that no studies compared the iovera with active treatment, and none identified potential clinical benefits of repeat administration.

Another independent technology assessment organization⁴⁹ found mixed results, noting that it is unclear if the device reduces postoperative pain and opioid use or improves function and quality of life compared with standard care post TKA because available studies (two RCTs and three nonrandomized controlled studies), are all at high risk of bias and report conflicting results.

Pulsed Radiofrequency Denervation

An independent technology assessment organization⁷² found an overall very low quality and limited body of evidence insufficient to allow conclusions regarding the efficacy and safety of pulsed RF for chronic pain arising from the SI joint. That same organization⁷⁶ also performed a review of pulsed RF for chronic cervical spine pain, again noting an overall low-quality body of evidence, though those studies did find improved pain for up to 3 to 6 months. In a review of pulsed RF for chronic shoulder pain the same group⁷⁸ found some positive but low-quality evidence suggesting it is safe and may improve pain and function, though further note that substantial uncertainty remains regarding comparative effectiveness versus alternative therapies, long-term outcomes and patient selection criteria.

Radiofrequency Ablation for Coccygodynia (Coccydynia)

An independent technology assessment organization⁷⁴ found the quality of evidence to be very low, citing limitations of the individual studies, a small number of comparative studies and a lack of experimental studies. Overall quality of the evidence is based on the balance of benefits and complications and is assessed taking into consideration the quality of the individual studies and the applicability of the data to general practice. All studies are retrospective and observational. The evidence base is comprised of 3 poor-quality studies and 1 very poor-quality study. Limitations of the individual studies also include insufficient follow-up time to determine some long-term outcomes.

Radiofrequency Ablation for Hip, Knee or Pelvic/Pelvis Osteoarthritis/Pain

Hunt et al⁴⁰ notes there is a far less robust body of literature to guide clinicians in the option of RFA for chronic hip pain. Although the authors report on a systematic review showing a high success rate following the procedure, it was noted that none of the studies were RCTs. The authors went on to note there is little data to guide estimates of rates of adverse events (AEs) but reported AEs following hip RFA include hypoesthesia and hematoma, particularly at the target for the accessory obturator nerve, illustrating the importance of using caution and carefully identifying the femoral artery prior to needle entry.

In the report for RF nerve ablation for the treatment of KOA, an independent technology assessment organization⁷⁹ concludes that a low-quality evidence base suggests that RFA of the genicular nerves may result in improvements in pain and function in patients with treatment-refractory pain associated with KOA; however, substantial uncertainty exists as to the consistency of clinically significant improvements in pain and the duration of effect of RFA on KOA-related pain. In addition, few studies evaluated the effect of RFA of the genicular nerve on QOL, and a standardized treatment protocol is lacking.

Radiofrequency Ablation for Post Herniorrhaphy Groin Pain

An independent evidence-based guideline organization¹⁰⁹ reports that the outcomes of nerve ablation are less favorable than surgical nerve excision, as nerve ablation only destroys the offending nerve ending(s), and recurrent pain may develop after subsequent nerve regeneration.

Radiofrequency Ablation for Shoulder Pain (Suprascapular Nerve)

Hunt et al⁴⁰ reports an insufficient quantity of literature establishing the effectiveness and safety of RFA for shoulder pain, as well as varied study methodologies such that no definitive conclusions can be drawn. The authors also note poor quality of evidence supporting long-term efficacy but reassuring safety profiles for pulsed RFA targeting the suprascapular nerve, concluding that additional research is needed to support the long-term efficacy and safety for RFA for shoulder pain.

An independent technology assessment organization⁷⁸ concludes that a low-quality of evidence base suggests that pulsed RFA may result in improvement in pain and function, but substantial uncertainty also exists related to variability of treatment protocols, patient populations, clinical significance of treatment benefit and long-term follow-up. Additional comparative evidence is needed to determine if it is a safe and effective treatment for shoulder pain.

Radiofrequency Ablation of the Dorsal Root Ganglia

An evidence-based clinical resource,¹¹⁰ in a review of a meta-analysis, notes that while RFA may provide improvement in pain scores in the short-term, the improvement was not sustained at six months and there was no difference in average disability at any time point during follow-up. Additional limitations include heterogeneity of patient selection and epidural injection content limits generalizability of findings.

Radiofrequency Ablation of the Genicular Nerve

The American Academy of Orthopaedic Surgeons³ makes a limited recommendation for knee denervation therapy for patients with symptomatic knee osteoarthritis, based on 6 studies comparing a variety of denervation methods (cryoneurolysis, intra-articular botulinum toxin and radiofrequency ablation) with placebo or an alternate therapy and one study comparing 2 radiofrequency techniques. The authors note that inconsistent evidence and bias in the included studies led to the limited recommendation.

An independent technology assessment organization⁷⁹ reports a low-quality evidence base suggests that RFA of the genicular nerves may result in improvements in pain and function in patients with treatment-refractory pain associated with knee osteoarthritis (KOA). Substantial uncertainty exists as to the consistency of clinically significant improvements in pain and the duration of effect of RFA on KOA-related pain. In addition, few studies evaluated the effect of RFA of the genicular nerve on QOL. Hayes noted that uncertainty remains for the use of RFA due to the lack of standardization in procedural techniques, and there are currently no established treatment guidelines that recommend the use of RFA for KOA. Additionally, the effects of RFA in general are temporary and generally limited to 1 year, as pain signal transmission will return with peripheral nerve regrowth and regeneration.

An independent evidence-based guideline organization⁹⁶ notes that the evidence regarding RFA of the genicular nerve is insufficient, conflicting, or poor and demonstrates an incomplete assessment of net benefit vs harm; additional research is recommended.

An evidence-based clinical resource¹⁰⁶ notes that while results with RFA of the genicular nerve appear promising, most studies are small and direct comparisons between techniques are not available. Further sham-controlled trials with larger sample sizes and longer follow-up are warranted.

Radiofrequency Ablation of the Pudendal Nerve

An independent technology assessment organization⁵⁶ identified a very limited number of publications regarding the use of pudendal nerve RFA; all are of small sample sizes and consist of case series or conference abstracts. The organization concludes that without a concurrent control group, the relative effectiveness and safety of RF denervation for PN compared to other treatment modalities cannot be fully determined. Sham-controlled trials are needed to assess the treatment's relative efficacy.

Radiofrequency Ablation of the Sural Nerve

Abd-Elseyed et al¹ review the use of RFA in the treatment of sural neuralgia after failure of conservative treatment, finding improvement in pain and function, noting that it may be a safe and effective treatment option. The authors caution, however, that further studies are needed to elucidate its effectiveness and safety profile in a larger population.

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Change Summary

01/01/2024 New Policy
08/06/2024 Annual Review, Coverage Change. Updated Coding Information
12/10/2024 Update, No Coverage Change Updated Coding Information
10/01/2025 Annual Review, Coverage Change. Updated Coding Information
03/02/2026 Annual Review, No Coverage Change
04/08/2026 CMS document update required
04/28/2026 CMS document update required
07/15/2026 CMS document update required
08/06/2026 CMS document update required